



ADVANCE TRAUMA

Trial Management Group

Meeting

March 3, 2026

Present

Martin Gedin Wärnberg, Chair (MGW)
Johanna Berg (JB)
Prashant Kharat (PK)
Rajdeep Singh (RS)
Kapil Dev Soni (KDS)
Anurag Mishra (AM)
Debojit Basak (DB)
Li Felländer-Tsai (LFT)

Apologies

Vivekanand Jha (VJ)
Girish Bakshi (GB)
Shamita Chatterjee (SC)
Abhinav Bassi (AB)
Karla Hemming (KH)
Anna Olofsson (AO)
Nobhojit Roy (NR)
Lovisa Strömmer (LS)
Monty Khajanchi (MK)

§ 1 Agenda

- Batch 1 patient inclusion (Table 1, next page)
- Batch 1 data review (Table 2, page 3)
- Batch 2 progress update (Table 3, page 4)
- Batch 3 progress update
- Tentative agenda for trial meeting in Mumbai on March 21 (separate attachment)
- Next meeting
- Any other business

§ 2 Batch 1 patient inclusion

We discussed inclusion of participants in the first batch. This first batch will complete including patients now in March, and follow up will continue until June. Some clusters have lower numbers of included participants, but overall we are meeting the target of including an average of 12 participants per cluster and period for the primary outcome. For the secondary outcomes the numbers have been smaller than expected and it might be that we will not achieve adequate sample sizes for these outcomes. It has been challenging to get accurate estimates from clusters on the expected volumes of eligible

participants, or at least the estimates that we have received have not always reflected the actual number of participants that we can include.

§ 3 Batch 1 data review

We reviewed the summary data for the first batch. The large variation in the number of injuries recorded per site has remained throughout this batch and we will need to be more vigilant with regards to this in the coming batches. JB asked whether we can include ISS in the summary data and the trial team will look into this. This would require us to code the injuries using AIS either manually or using some automatic solution, which would involve the database team as well. We will follow up on this. There is also substantial variation in the primary outcome, ranging from <5% to >20%. We did expect variation in the primary outcome because of the differences in case mix between sites, but with an average of 16% driven by one center it is unlikely that we will see an average of 20% (which we assumed) in future batches. We did however calculate the sample size required to detect a reduction in the primary outcome from 10 to 7.5% and with an average per cluster and period volume of 20 participants we have > 80% for this difference. In the first batch the average per cluster and period volume is ~20, so we should be able to detect a difference at this level as well.

§ 4 Batch 2 progress update

Batch 2 is underway and has entered the third study period. There is less variation in participant volumes in this batch so far, which is good.

§ 5 Batch 3 progress update

We are on track to start the third batch by mid May, which is the target. The processes with ethics and clinical trial agreements are underway. LFT asked how the ethical review processes have been so far and in general it has been straightforward with few comments. The processes with the clinical trial agreements have been more involved.

§ 6 Tentative agenda for trial meeting in Mumbai on March 21

There will be a trial meeting for investigators and clinical research coordinators in Mumbai on March 21st. The tentative agenda for this meeting was circulated before this meeting but there were no additional inputs on the agenda. There will be a hybrid option and the TMG members available will have an opportunity to meet on the 22nd.

§ 7 Next meeting

The next TMG meeting is on March 22, in person for those in Mumbai. The next online meeting is May 11.

§ 8 Any other business

There were no additional points for discussion.

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Table 1. Batch 1 patient inclusion

Cluster	Cohort	Period 1 Mar 25	2 Apr 25	3 May 25	4 Jun 25	5 Jul 25	6 Aug 25	7 Sep 25	8 Oct 25	9 Nov 25	10 Dec 25	11 Jan 26	12 Feb 26	13 Mar 26
CMC	All	12	03	03	18	12	12	12	--	08	09	13	07*	
	QoL/Dis	NA	03	03	17	11	12	11	02	NA	NA	NA	NA	
	ATLS	NA	02	03	14	09	10	08	01	NA	NA	NA	NA	
Cooper	All	13	11	10	05	06	12	08	13	10	14	15	13*	
	QoL/Dis	NA	NA	05	03	03	07	05	07	NA	NA	NA	NA	
	ATLS	NA	NA	04	01	02	03	02	03*	01	NA	NA	NA	
GMC	All	04	12	33	32	30	21	39	24	26	24	27*	21*	
	QoL/Dis	NA	NA	NA							17	NA	NA	
	ATLS	NA	NA	NA	12	09	03	06	14	08	05	NA	NA	
SSKM	All	47	66	58	48	59	50	46	53	52	57	52	35*	
	QoL/Dis	NA	NA	NA	NA	45	39	38	46	38	41	--	NA	
	ATLS	NA	NA	NA	NA	18	19	24	22*	09	11	11	NA	
HIMS	All	20	27	29	26	21	28	12	20	18	12	15*	13*	
	QoL/Dis	NA	NA	NA	NA	NA	13	08	11	11		--		
	ATLS	NA	NA	NA	NA	NA	05	03	07	05	04	05*	02*	

Note: Bold numbers indicate periods where the target of 12 participants have been met for that cluster and period. *Indicate that the numbers are not final and might be updated.

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Table 2. Batch 1 summary data

Characteristic	Cooper (n = 114)	HIMS (n = 193)	SSKM (n = 637)	CMC (n = 57)	GMC (n = 166)	Overall (n = 1167)
Total Injuries	322	512	2524	94	217	3659
Age in years, median (IQR)	35 (24-47)	38 (26-54)	39 (26-55)	34 (25-42)	35 (24-48)	37 (25 to 53)
Missing no.	1 (0.9%)	NA	NA	NA	1 (0.6%)	2 (0.2%)
Female sex, no. (%)	16 (14.0%)	40 (20.7%)	116 (18.2%)	12 (19.6)	21 (12.7%)	205 (17.6%)
Missing no.	1 (0.9%)	NA	NA	NA	1 (0.6%)	2 (0.2%)
Mechanism of injury, no. (%)						
Transport	53 (49.1)	97 (51.2)	406 (65.7)	33 (57.9)	79 (53.7)	668 (59.7)
Fall	31 (28.7)	70 (37.2)	157 (25.4)	8 (14.0)	28 (18.1)	294 (29.3)
Assault	19 (17.6)	6 (3.2)	14 (2.3)	8 (14.0)	25 (17.0)	72 (6.4)
Self-harm	0 (0)	11 (5.9)	3 (0.5)	2 (3.5)	3 (2.0)	19 (1.7)
Other	3 (2.8)	4 (2.1)	38 (6.1)	6 (10.5)	5 (3.4)	56 (5.0)
Missing	2 (1.9)	NA	NA	NA	7 (4.8)	9 (0.8)
Transfer, no. (%)	18 (16.2%)	60 (31.1%)	485 (76.1%)	6 (10.5)	133 (81.1%)	702 (60.4%)
SBP, median (IQR)*	110 (100-120)	120 (110-140)	127 (116-138)	120 (110-140)	125 (111-140)	124 (110- 139)
Missing, no.	4 (3.5%)	3 (1.6%)	44 (6.9%)	4 (7.0%)	6 (3.6%)	61 (5.2%)
RR, median (IQR)*	22 (19-24)	19 (18-20)	20 (18-21)	21 (20- 22.5)	22 (20-24)	20 (18 – 22)
Missing, no. (%)	5 (4.3%)	3 (1.6%)	68 (10.7%)	13 (22.8%)	8 (4.8%)	93 (8.3%)
SpO2, median (IQR)*	98 (97-99)	98 (96-99)	99 (97-100)	98 (97-100)	97 (92-98)	98 (96 – 100)
Missing, no. (%)	4 (3.5%)	2 (1.0%)	16 (2.5%)	1 (1.8%)	12 (9.6%)	45 (3.9%)
GCS, median (IQR)*	15 (11-15)	15 (6-15)	15 (13-15)	15 (13-15)	15 (15-15)	15 (13 – 15)
Missing, no. (%)	11 (9.6%)	NA	9 (1.4%)	2 (3.5%)	9 (5.4%)	31 (2.7%)
CT, no. (%)	86 (33.0%)	350 (32.0%)	667 (30.6%)	46 (80.7)	169 (35.7%)	1318 (32.0%)
Surgery, no. (%)	18 (17.8%)	68 (33.5%)	354 (51.4%)	16 (28.1)	69 (41.6%)	525 (43.1%)
ICU admission, no (%)	72 (91.1%)	16 (16.3%)	271 (42.2%)	11 (19.3)	7 (4.4%)	377 (36.8%)
Hospital disposition, no. (%)						
Alive	96 (88.1%)	175 (90.7%)	470 (76.4%)	54 (94.7)	149 (93.7%)	944 (83.3%)
Dead	12 (11.0%)	18 (9.3%)	143 (23.3%)	3 (5.3)	5 (3.1%)	181 (16.0%)
Transfer	1 (0.9%)	NA	2 (0.3%)	NA	3 (1.9%)	6 (0.5%)
Missing	8 (6.8%)	12 (5.9%)	27 (4.2%)	NA	8 (4.9 %)	55 (4.7%)

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Table 3. Batch 2 patient inclusion

Cluster	Cohort	Period 1	2	3	4	5	6	7	8	9	10	11	12	13
		Dec 15- Jan 15/26	Jan 16- Feb 15/26	Feb 16- Mar 15/26										
Dibrugarh	All	07	30	07*										
	QoL /Dis	NA	NA	NA										
	ATLS	NA	12	NA										
Holy Family	All	25	29	13*										
	QoL /Dis	NA	NA											
	ATLS	NA	01*	06*										
KEM	All	09*	25*	05*										
	QoL /Dis	NA	NA	NA										
	ATLS	NA	NA	NA										
Dayanand	All	28	31	06*										
	QoL /Dis	NA	NA	NA										
	ATLS	NA	NA	NA										
Sion	All	20	15	09*										
	QoL /Dis	NA	NA	NA										
	ATLS	NA	NA	NA										

Note: Bold numbers indicate periods where the target of 12 participants have been met for that cluster and period.

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